



EMyU: Youth Custodial Patient Management SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division East Metropolitan Youth Unit (EMyU) Inpatient Service	Medical, Nursing, Allied Health, Security, External Security

Purpose

The purpose of this standard operational procedure (SOP) is to outline processes for the movement of youth custodial patients from the Banksia Hill Detention Centre (BHDC) (Department of Justice) to the EMyU (the Parties). A Memorandum of Understanding (MOU) has been signed (Oct 2023) between the Parties to ensure that all policy and procedural requirements of both Parties are adhered to. The Company transferring the youth custodial patients on behalf of the Parties will adhere to the MOU.

Key alerts

Staff are to ensure appropriate balance should be maintained between the demands of patient care and custody. Security measures should be commensurate with the assessed security risk of the patient and should not compromise the quality of clinical care.

Guiding principles

There are guiding principles fundamental to the relationship between health and the custodial system that apply to healthcare.

- The Mental Health (MH) Division is committed to the safety and duty of care to its patients, staff, the general public and the guards/officers accompanying patients.
- The MH Division supports the use of least restrictive practices to be undertaken in the context of trauma informed care principles,¹ the Mental Health Act (MHA) 2014 and the [Charter of Mental Health Care Principles \(MHA 2014\)](#).
- All information which is not in the public domain will be maintained as confidential by the Parties.
- Access, confidentiality and consent for youth custodial patients to healthcare should be the same as for the general population.
- The Parties will act in good faith in exercising their responsibilities and obligations and agree to work collaboratively.
- Health care and security provided is based on the clinical standards/procedures/policies, including:
 - Standard Guidelines for Corrections in Australia (clause 4.1)² and the Standards for Juvenile Custodial Facilities (clause 4.2)³
 - BHDC Operational Policy and Procedures Framework⁴ and other, relevant Department of Justice policies and procedures and the Department of Health.

Links to relevant documents

- Refer to RPBG [EMyU hub page](#)

Procedure details

Prior to the transfer of the youth custodial patient to the EMyU:

Referral

EMyU staff i.e. Nurse Unit Manager (NUM) or delegate (Clinical Nurse/Shift Coordinator/Duty NUM/After Hours Clinical Nurse Specialist [CNS]) will:

- Ensure receipt of completed BHDC Security Risk Assessment and that it is inclusive of:
 - Youth custodial patient's name
 - Gender/sexuality identity/cultural diversity needs
 - Date of birth
 - Estimated time of arrival at the EMyU
 - Security rating
 - Absconscion risk status (high, medium, low)
 - Number of guards/officers in attendance
 - Guard/officer ID number(s)
 - Restraints used
 - Any intended restriction on visitors
 - Any other requirements (e.g. wheelchairs, requirements for transmission-based precautions – including reasons, etc)
- Appraise the referral information and discuss with the Consultant/On Call Consultant who will make the decision whether to admit the patient
- Inform the referrer of the outcome of the referral.

Procedure details cont'd

On arrival/admission

The EMyU nursing staff are to:

- Inform Security of the expected admission (NUM/delegate) and via Computer Assisted Radio Personnel System (CARPS)
 - For BHS interhospital transfer of **low and medium-acuity patients**, use the [Patient Transfer Coordination Hub](#) (PaTCH) booking system
- Meet the guards/officers at the ambulance bay
- Orientate to EMyU and discuss roles and responsibilities and agreed initial plan/approach (by Shift Coordinator)
- Admit patients received from BHDC to the EMyU Psychiatric Intensive Care Unit (PICU) while awaiting review by a Consultant Psychiatrist/On Call Consultant
 - Refer to **MH Inpatient Triage to Discharge SOP**
- Provide clear and timely communication to ensure effective clinical handover. Refer to **RPBG Clinical Handover Policy**
- Complete an initial assessment and all documentation and reporting requirements
- Use de-escalation or alternative coping strategies for patients who are agitated/aggressive and/or if there are critical incidents escalate to the Consultant Psychiatrist/On Call Consultant. For guidance, refer to the **Prevention and Management of Aggressive Behaviours within MH Inpatient Setting SOP**
- If a conflict of interest arises at any time between the Parties and the rights, duties and responsibilities of either Party becomes compromised, the Parties will attempt to negotiate a resolution. If a resolution is not achieved the matter is to be escalated to the NUM/Senior Nurse On Call/Consultant Psychiatrist/On Call Consultant
- Ensure that under no circumstances are BHDC guards to film patients (audio and/or visual recordings) whilst in RPBG care.

Visits

EMyU staff are to ensure:

- **Visits to patients are only permitted following completion of formal admission as mandated and managed by BHDC staff**
- Support patient's access and/or use of mobiles or personal electronic devices as mandated and managed by the BHDC staff, e.g. lists of persons who are prohibited to contact the patient, where relevant
- Guards/officers must be in attendance at all times during visits
- Visitors (i.e. patient's personal support persons/Guardians/family) adhere to visiting protocols as per the **RPBG Visitor Management Policy**.

Compliance monitoring

Compliance with this document will be monitored by

- A review of clinical incidents and/or Combined Hazard or Incident Report (CHoIR) reported monthly via the RPBG Occupational Safety and Health (OSH) Management Committee/relevant MH Division Clinical Quality & Safety meetings to identify any workplace safety related incidents attributed to Youth Custodial Patients
- Compliments/Complaints Management System.

Facilitator

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East Metropolitan Health Service (EMHS) Aboriginal Health Strategy Team

Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the EMyU Youth Custodial Patient Management SOP through consultation with the Aboriginal Health Strategy Team and Aboriginal Health Liaison Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 553).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

References

1. What is Trauma informed Care? 2015. University at Buffalo. [Cited May 2021]. Available from URL: <http://socialwork.buffalo.edu/social-research/institutes-centers/institute-on-trauma-and-trauma-informed-care/what-is-trauma-informed-care.html>
2. Government of Western Australia. (2012). *Standard Guidelines for Corrections in Australia*. Department of Corrective Services. Perth, Western Australia.
3. Department of Juvenile Justice. (1999). *Australasian Juvenile Justice Administrators; Standards for Juvenile Custodial Facilities*. New South Wales, Sydney.
4. Department of Justice. (2020). *Banksia Hill Detention Centre Operational Policy and Procedures Framework*. New South Wales, Sydney.

Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Clinical Handover Policy](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Mental Health Restraint and Seclusion Policy](#)
- [Sexual Safety Policy](#)
- [Security Request for External Security Staff SOP](#)
- [Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [EMyU hub page](#)
- [Prevention and Management of Aggressive Behaviours within MH Inpatient Setting SOP](#)
- [Visitor Management Policy](#)
- MH Inpatient Triage to Discharge SOP

EMHS Policy Hub

- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [Rights of Patients in Mental Health Policy](#)
- [Security Policy](#)
- [Patient Transfer Coordination Hub](#)

Department of Health Policy Hub

- [State-wide Standardised Clinical Documentation \(SSCD\) Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- EMHS' EMyU and Banksia Hill Detention Centre MOU 2023
- [Mental Health Act 2014](#)
- [Work Health And Safety Act 2020](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)

Legislative requirements, the evidence and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy HUB.

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 1: Clinical Governance

Standard 2

Standard 10: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010):

Standard 1: Rights and responsibilities

Standard 2: Safety

Standard 3: Partnering with Patients

Standard 10: Delivery of care